PATIENT DEMOGRAPHIC FORM 10/27/2026

YANEZ FREITEZ, ROSANGELA ANA

DOB: 07/09/1986
Sex: Female
DOA: 09/07/2026
Case type: No Fault
Address: 725 10TH ST, New York, NY, 10019

Cell Phone: 347-758-1902
Home phone:
Social Security Number:

Primary Insurance: PROGRESSIVE (NY) - NYP
Address: PO BOX 2930, Clinton, IA, 52733
Claim Number: 26-599343106
Policy Number: 875571455
Policy Holder: YANEZ FREITEZ, ROSANGELA ANAIS
Phone Number:
Fax:
Carrier Case:
WCB Case:

Secondary Insurance:
Address:
Claim Number:
Policy Number:
Policy Holder:
Phone Number:
Fax:

BI Attorney: MORRISON & WAGNER LLP
Address: 49 W 38TH STREET, New York, NY, 10018
Phone number: 212-343-8000
WC Attorney:
Address:
Phone number:

Referring Doctor:

Referring Office:

ATLANTIC MEDICAL & DIAGNOSTIC, P.C.

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PROGRESS NOTE

Patient First Name:	Patient Last Name:	Date of Birth:	Sex:
Rosangela	Yanez Freitez	07-09-1986	Female
Patient Address:		**SSN:**	
725 10th Street, New York NY 10019			
Attending Provider:	**Referring Provider:**	**Visit Date:**	**Chart No.:**
Jerry Samedi, PA-C		09-16-2026	SCL74562
Insurance Name:	**Insurance Subscriber's ID:**	**Date of Injury:**	**Claim Id:**
Progressive Insurance Co-NF	875571455	09-07-2026	NO NUMBER
Appointment Location Address:	**Encounter Type:**		
41-12 77th Street , 3rd Floor, Elmhurst NY 11373 - 1996	New Patient - Auto-Accident		

Chief Complaint: Headache, Cervical Spine Pain, Shoulder Pain, Thoracic Spine Pain, Knee Pain, Lumbar Spine Pain

Motor Vehicle Accident

Rosangela Yanez Freitez is a 40 year old female who was involved in an accident on 09-07-2026. Patient current employment status is Cook.Working Reduced hours due to accident At the time of accident patient was wearing seatbelt. Patient was a back seat passenger. Patient was in the car. Three other people were in the vehicle. she suffered a rear impact to the vehicle. moving she suffered no loss of consciousness.
This was the first car accident of the patient. She was immediately admitted to the ER. Brooklyn Hospital Center In the ER, a physical exam, X-rays and pain killers was done. Cray for Neck, Shoulder and Wrist was then sent home
Patient does not have history of back problems in the past.

Headache

She states that her headache is constant. Location of headache at onset was Frontal and Bilateral. Patient reports that she has same episode(s) 3-6 hours. **She reports that she has photophobia.** There is no vomiting associated with her headache. **There is neck stiffness.** She does not report pain in her eyes.

Cervical Spine Pain
Patient reported pain is bilateral. The patient describes her pain as constant. The pain is aching, pressure like, pulling and tight . Patient says, at its worse her pain is 8/10. Right now she describes her pain as 8/10 on a pain scale of 0-10. The pain is made worse by bending, such as cleaning, moving items, laundry, movement, sitting a long time and turning side to side. Other associated symptoms/problems are as follows: difficulty staying asleep due to pain .

Shoulder Pain
Patient reported pain is on right side. The patient describes her pain as constant. The pain is aching, numbing, pressure like, sharp, pulling and tight . **The pain radiates to the right upper extremity. Numbness** Patient says, at its worse her pain is 9/10. Right now she describes her pain as 9/10 on a pain scale of 0-10. The pain is made worse by bending, such as cleaning, moving items, laundry, lifting, movement and laying on area. Other associated symptoms/problems are as follows: difficulty staying asleep due to pain .

Thoracic Spine Pain
Patient reported pain is bilateral. The patient describes her pain as constant. The pain is aching, pressure like, sharp, pulling and tight . Patient says, at its worse her pain is 8/10. Right now she describes her pain as 8/10 on a pain scale of 0-10. The pain is made worse by bending, such as cleaning, moving items, laundry, lifting, movement and standing a long time. Other associated symptoms/problems are as follows: difficulty staying asleep due to pain .

Knee Pain
Patient reported pain is on left side. The patient describes her pain as constant. The pain is aching, pressure like, sharp, tight and feels weak . Patient says, at its worse her pain is 9/10. Right now she describes her pain as 9/10 on a pain scale of 0-10. The pain is made worse by bending, going up stairs, going down stairs, such as cleaning, moving items, laundry, movement, standing a long time and standing straight up. Other associated symptoms/problems are as follows: difficulty staying asleep due to pain .

Lumbar Spine Pain
Patient reported pain is bilateral. The patient describes her pain as constant. The pain is aching, pressure like, sharp, pulling and tight . Patient says, at its worse her pain is 9/10. Right now she describes her pain as 8/10 on a pain scale of 0-10. The pain is made worse by bending, such as cleaning, moving items, laundry, lifting, sitting a long time, standing a long time and standing straight up. Other associated symptoms/problems are as follows: difficulty staying asleep due to pain .

Past Medical History
No Known Past Medical History

Surgical History
Tummy Tuck: 2026
Breast Augmentation : 2026
NO METALS

Allergy
No Known Drug Allergies.

Social History
Use of Drugs/Alcohol/Tobacco: Patient states that she never drinks any alcohol. Current some day smoker

Review of Systems

Musculoskeletal: Reports **muscle pain**, **muscle cramp**, **muscle pain or tenderness**, **neck pain**, **shoulder pain**, **back pain** and **joint pain**. Denies muscle twitches, muscle wasting, muscle weakness, loss of muscle bulk, joint stiffness, joint swelling, morning stiffness, swollen joints, night cramps, atrophy and postural abnormalities. Denies any joint problem or abnormalities. Denies any limitation of joint movements. No history of fractures.

Neurological: Denies seizures, blackouts, trouble with memory, trouble concentrating, stroke, loss of strength, fainting spells, memory loss, involuntary movements, poor coordination, numbness, spasticity, weakness and tremors. The patient does not observe any gait disturbance. Reports **headache**.

Psychiatric: Denies anxiety, depression and mood swings. Reports **nervousness** and **sleeping difficulty**.

Vitals

Weight: 168.00 lbs. **Handedness:** right-handed. **Height:** 66.00 inches.

Physical Examination

Cervical Spine Exam: Inspection reveals normal curvature of the cervical spine. The cervical spine musculature is supple. Palpation of the cervical facets reveals no pain. Cervical spine is noted to be stable. **Anterior flexion is noted to be 30 degrees (ROM range 0-60 degrees). There is pain noted when neck is flexed anteriorly. Extension of cervical spine noted to be 40 degrees (ROM range 0-75 degrees). There is pain noted with extension of cervical spine. Left lateral rotation noted to be 50 degrees (ROM range 0-80 degrees). There is pain noted with left lateral rotation of C-spine. Left lateral flexion noted to be 20 degrees (ROM range 0-45 degrees). There is pain noted with left lateral flexion of C-spine. Right lateral rotation of the C-spine is noted to be 40 degrees (ROM range 0-80 degrees). There is pain noted with right lateral rotation. Right lateral flexion of the C-spine is noted to be 20 degrees (ROM range 0-45 degrees). There is pain noted with right lateral flexion.** There are no palpable trigger points in the muscles of the head and neck.

Thoracic Spine: There is no evidence of atrophy or asymmetry noted in the thoracic spine. **There is tenderness noted at paraspinal muscles, facet joint lines, levator scapulae muscle and supraspinatus.** Range of motion of the thoracic spine is normal with both flexion and extension without pain. There is no evidence of crepitation, laxity or instability noted. **Hyperextension of thoracic spine caused increased pain.**

Lumbar Spine: Inspection of the lumbar spine reveals no scoliosis. Palpation of the lumbar facet reveals no pain. There is no pain noted over the lumbar intervertebral spaces (discs) on palpation. Palpation of the bilateral sacroiliac joint area reveals tenderness Palpation of the bilateral sacroiliac joint area reveals no pain. Trochanteric Bursa - Tenderness Palpation of the greater trochanteric bursa reveals no tenderness bilaterally. **Anterior flexion of lumbar spine is noted to be Pain at 45 degrees. Anterior lumbar flexion causes pain. Extension of lumbar spine is noted to be 5 degrees (ROM range 0-30 degrees). There is pain noted with lumbar extension. Left lateral flexion of the lumbar spine is noted to be 5 degrees (ROM range 0-25 degrees). Left lateral flexion causes pain. Right lateral flexion of the lumbar spine is noted to be 5 degrees (ROM range 0-25 degrees). There is pain noted with right lateral flexion. Trigger Point Tenderness Assessment: Erector Spinae muscle, Lumbar Multifidus Muscle, Latissimus Dorsi Muscle, Positive Jump Sign, Positive Local Twitch response and Positive Referred Pain.**

Joints-Shoulder: **On palpation, there is tenderness noted over the right anterior acromioclavicular joint, anterior acromial border, posterior glenoid rim, trapezius and supraspinatus. Right shoulder impingement: Positive. Right shoulder O'brien's test: Positive.** Right shoulder popeye sign: Negative.

Joints-Knee: **Tenderness is present in the vastus medialis, patella and posterior knee of the left knee.**

Neurology - Coordination: Diadochokinesis is found to be normal. Finger-to-nose testing is normal. The

patients gait appears to be normal. The patient was able to do heel walk. The patient was able to do toe walk.
Neurology - Straight Leg Raising: Right: Positive at 35 degrees. Left: Positive at 40 degrees.
Neurology - Motor Strength: Right UE strength - Flexors: 5/5. Left UE strength - Flexors: 5/5. Right UE strength - Extensors: 5/5. Left UE strength - Extensors: 5/5. Right UE Tone: Normal. Left UE Tone: Normal. Right LE strength - Flexors: 5/5. Left LE strength - Flexors: 5/5. Right LE strength - Extensors: 5/5. Left LE strength - Extensors: 5/5. Right LE Tone: Normal. Left LE Tone: Normal.

Pain Management

- Cluneal Nerve Block
- ManaSport+ Device
- Physical Therapy Prescription
- PNEUMATIC COMPRESSION DEVICE
- Sciatic Nerve Injection
- Transcutaneous Electrical Joint Stimulation Device
- Trigger Point injection

Labs Ordered
Order No: LAB0113138 Dated: 09-16-2026 Lab: DME VENDOR
DME: CERVICAL COLLAR, CERVICAL PILLOW, LUMBO SACRAL SUPPORT, LUMBAR BACK CUSHION, BED BOARD/EGG CRATE MATTRESS, HEATING PAD (THERMOPHORE), COLD/HOT WATER CIRCULATION UNIT
Order No: LAB0113139 Dated: 09-16-2026 Lab: DME VENDOR
DME: Orthopedic Positioning Seat

Radiology Ordered
Order No: RAD0055831 Dated: 09-16-2026 Rad: HIGHLINE RADIOLOGY
Magnetic Resonance Imaging: MRI Cervical Spine without CM, MRI Lumbar Spine without CM, MRI Thoracic without CM
Order No: RAD0055832 Dated: 09-16-2026 Rad: FAST CARE MEDICAL DIAGNOSTICS, PLLC
Magnetic Resonance Imaging: MRI Left Knee without contrast, MRI Right Shoulder without CM

Assessment and Plan
ICD: Cervicogenic headache (R51.9)
ICD: Acute cervical myofascial strain (S16.1XXA)
ICD: Acute thoracic myofascial strain (S29.019A)
ICD: Acute lumbosacral myofascial strain (S39.012A)
ICD: Derangement of right shoulder joint (M24.9)
ICD: Derangement of knee, left (M23.92)

Diagnosis
ICD: Cervicogenic headache (R51.9)
ICD: Acute cervical myofascial strain (S16.1XXA)
ICD: Acute thoracic myofascial strain (S29.019A)
ICD: Acute lumbosacral myofascial strain (S39.012A)
ICD: Derangement of right shoulder joint (M24.9)
ICD: Derangement of knee, left (M23.92)
Elements of Medical Decision Making

Assessment: Lumbar Posttraumatic Sprain and Strain Syndrome, Lumbar disc displacement
Patient presents in the office with worsening symptoms of lower back pain. Patient is status post trauma/motor vehicle accident. Patient was asymptomatic prior to the accident.
MRI: PENDING R/O derangement
EMG/NCV: PENDING R/O Nerve compression
As a direct result of the injury patient also endorses difficulties with activities of daily living and a negative impact on functional capacity and range of motion.
Ultrasound of lumbar spine conducted using Butterfly IQ+ transducer.
Muscle patterns of lumbar spine are consistent with diffuse and focal echogenic deflections at paraspinal musculature. This pattern is highly suggestive of muscle spasms of musculature at bilateral L4-L5, L5-S1.

Therapeutic Pain Management injections (ie: trigger point injections, etc) shortly after a motor vehicle accident are being recommended because they are medically necessary and extremely helpful as detailed below.

When a patient experiences whole body trauma associated with a motor vehicle accident, their bodies are subjected to physical forces of acceleration and deceleration which are transmitted throughout their entire body, and typically manifest as more problematic in those areas which are more vulnerable. This often includes the spine, along with shoulder, knee and wrist joints and the muscles and other soft tissues (ligaments, tendons and cartilage) which are in close proximity to these areas.

According to the Cleveland Clinic, many providers use trigger point injections alongside physical therapy and stretching exercises to alleviate myofascial pain. There are significant benefits to speed up recovery by initiating TPI therapy as early as possible after injury or trauma. This strategy is particularly beneficial for those patients who are unable to perform effective physical therapy or stretching exercises due to intense pain and helps amplify and expedite the impact of conservative care. Many of our patients are dealing with acute worsening or persistent lower back, neck and or extremity pain. A motor vehicle accident or mechanical trauma is often deadly or life threatening. After an injury the body's enormous demand to drive forward the healing process is largely hindered by the patient's limited range of motion with body mechanics, acute pain and stiffness. The sequela of these symptoms is further exacerbated by the patients inability to perform and carry out the necessary requirement of physical therapy and stretching exercises. If these steps mentioned above are not initiated as early as possible, the patient is at a higher risk for developing scar tissue and adhesions within the injured muscle groups and tendons. This often leads to permanent decreased range of motion, difficulty with activities of daily living, human suffering and increased disability. Patients who do not receive prompt intervention often end up having a higher incidence of surgical intervention, which we are trying to avoid.

Deploying TPI therapy as early as possible will yield the best possible outcome for recovery utilizing the least number of resources. The efficacy of trigger point injections is intensified when combined with conservative care. This multidimensional pain management technique is often beneficial to decrease both the need and use of strong habit-forming oral pain medications such as Oxycodone, Oxycontin and Morphine.

Plan
- RTC 4 weeks, review TPI/NBI Therapy and plan of care.
- Patient verbally expressed understanding The Following plan has been discussed with the patient in detail:
- U/S guided Trigger point injection therapy to be performed to improve functional capacity and mitigate symptoms of pain to increase capacity for PT/exercises.
- Patient would benefit from consultation for Cervical and Lumbar EMG/NCV testing to ensure no significant nerve damage is present.
- Patient would benefit from functional capacity testing and range of motion testing to gauge progress/improvement.

- MRI without contrast of the Cervical, Thoracic, Lumbar, R Shoulder, L Knee regions Ordered.

- Patient will continue Chiropractic, Physical Therapy, Acupuncture 3 to 4 times/week for 4-6 weeks.

-Psychiatric evaluation requested in an effort to diagnose and treat persistent HA, anxiety, depression or sleep disorders.

Rx Prescribed today:
-cyclobenzaprine 7.5 mg tablet 1 Tablet Once A Day PRN for 30 Days
-diclofenac 3 % topical gel
-lidocaine 5 % topical ointment
-Ibuprofen 800 mg oral tablet

The following DME's were ordered:
-ManaSport+ Device
-PNEUMATIC COMPRESSION DEVICE
-Transcutaneous Electrical Joint Stimulation Device

-CERVICAL COLLAR
-CERVICAL PILLOW
-LUMBAR BACK CUSHION
-LUMBO SACRAL SUPPORT
-BED BOARD
-EGG CRATE MATTRESS
-HEATING PAD (THERMOPHORE)
-COLD/HOT WATER CIRCULATION UNIT
-Orthopedic Positioning Seat

Referral sent for functional capacity test, range of motion , acupuncture , chiropractor. Neurology, orthopedic , interventional management as indicated

- RTC 4 weeks, review TPI, IAI, NBI Therapy and plan of care.
- Patient verbally expressed understanding and verbally agreed to plan.

Procedure
TODAYS VISIT: TYPE: (Ultrasound Guided Trigger Point Injection Therapy Lumbar Spine)
Procedure/Interventions
Bilateral CLUNEAL NERVE BLOCK injection was performed on this visit.
- I prepped and filled syringe with 1cc Lidocaine, 0.5cc Bupivacaine, 0.5cc Dexamethasone and administered to the areas listed above.
- Informed Consent obtained
- Risks of NBI discussed with patient, including but not limited to infection and allergic reaction discussed
- Sterile prep and 30G needle were used.
- Patient tolerated procedure well and endorses pain relief, no complications.
- Will continue to manage and follow consistently, will evaluate and assess for level of improvement on upcoming visits.

TODAYS VISIT: TYPE: (Ultrasound Guided Trigger Point Injection Therapy Lumbar Spine)
Procedure/Interventions

Bilateral Erector Spinae muscles Ultrasound Guided Trigger Point injection was performed on this visit.
- I prepped and filled syringe with 1cc Lidocaine, 0.5cc Bupivacaine, 0.5cc Dexamethasone and administered to the areas listed above.

- Informed Consent obtained
- Risks of TPI discussed with patient, including but not limited to infection and allergic reaction discussed
- Sterile prep and 30G needle were used.
- Patient tolerated procedure well and endorses pain relief, no complications.
- Will continue to manage and follow consistently, will evaluate and assess for level of improvement on upcoming visits.

TODAYS VISIT: TYPE: (Ultrasound Guided Sciatic Nerve Block Injection Therapy)
- Informed Consent obtained
- Sterile prep and 30G needle were used.
- Patient tolerated procedure well and endorses pain relief.
-U/S guided Bilateral Sciatic Nerve Block Injection

Medications used:
1cc of lidocaine, 0.5cc of bupivacaine, 0.5cc of dexamethasone

Multiple sites for trigger point injections are indicated due to the complex nature of whole-body trauma and the body's natural tendency to resist the applied forces during an accident. Clinically significant trigger points or knots are identified with ultrasound guidance carefully noting echogenic deflections within specified muscle groups that are so often characteristic of muscle spasms. Muscles that are in proximity or adjacent need to be treated due to physical mechanics innervation and function of these muscle groups. Patients may receive up to six trigger point injections, although many suffer from more than ten clinically significant myofascial knots that would clearly benefit from treatment.
The diagnosis of trigger points is not subjective, and their presence is confirmed two different ways, by physical exam and by ultrasound. Diagnosis of a trigger point is NOT adequate by itself. One must know the precise anatomic location so that medication can be delivered by needle injection to exact area which requires treatment in a safe and effective way.

Safety: Many therapeutic injections are performed in areas with associated nerves, arteries or veins. Avoiding these structures is very important from a safety perspective.

Efficacy: We utilize dexamethasone sodium phosphate, bupivacaine and lidocaine medications when performing therapeutic injections. If they are not introduced to the precise location, it will not be effective and can possibly create other problems. Visualization with ultrasound helps ensure clinical efficacy is optimized.

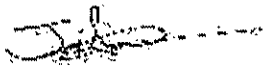
CPT/HCPC Codes:

Name	Code	Units	Comments
Office O/p New Mod 45 Min	99204	1.00	
Prolong Service W/o Contact	99358	1.00	
Njx 1/Mlt Trigger Point 1/2	20552	1.00	
Njx Aa&/strd Other Pn/branch	64450	2.00	
Njx Aa&/strd Sciatic Nrv Img	64445	2.00	
Injection Dexamethosone Sodium Phosphate 1 Mg	J1100	6.00	
Inj, bupivacaine, nos, 0.5mg	J0665	15.00	
Injection, Lidocaine 1 mg	J3490	30.00	
Echo Guide For Biopsy	76942	1.00	
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Prescription
lidocaine 5 % topical ointment Apply 2-3 grams to affected area every 8 Hours PRN Pain for 30 Days , Prescribe 250 Gram
diclofenac 3 % topical gel 2-3 Gram Every 8 Hours PRN for painful areas, may alternate with Lidocaine ointment, por dolor musculo, Prescribe 200 Gram
ibuprofen 800 mg tablet 1 Tablet Every 8 Hours FOR SYSTEMIC INFLAMMATION And Pain for 15 Days , Prescribe 90 Tablet
cyclobenzaprine 7.5 mg tablet Take 1 tablet by mouth once daily at bed time (MUSLE RELAXER-MAY CAUSE DROWSINESS) for 30 Days , Prescribe 30 Tablet

Follow up: 4 Weeks



Jerry Samedi, PA-C
NPI: 1104784701
This has been electronically signed by Jerry Samedi, PA-C on 09-16-2026.

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Moeschler SM, et al. Pain. 2021. PMID: 33433147 Clinical Trial.

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